Research Paper

"A study on customer awareness and adoption on Health Insurance among Educated Youth."

Submitted in partial fulfillment of the requirements of the

PGDM – AICTE Approved for the award of degree in

POST GRADUATE DIPLOMA IN MANAGEMENT

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REAL WORLD. REAL LEARNING.

2024-2026

STUDENT DECLARATION

I hereby declare that this report titled "**A study on customer awareness and adoption on Health Insurance among Educated Youth.",** submitted by me to ISBR Business School in partial fulfillment of requirements of PGDM program is a Bonafide work carried out by me under the guidance of **Prof. VN Lakshmi Prasad.** This has not been submitted to any other University or Institution for the award of any degree/diploma certificate or published any time before.

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Date:**23/05/2025** **Shrestha Sarkar**

Place: Bangalore PG24299

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BODY STRUCTURE OF THE STUDY

CHAPTER 1: INTRODUCTION

1.1 Background of the Study

Health insurance plays a crucial role in financial protection against rising medical expenses. In India, despite increasing literacy and digital exposure, the adoption of health insurance among educated youth remains inconsistent.

1.2 Problem Statement

Even though educated youth are aware of financial planning tools, there exists a gap between **awareness and actual adoption** of health insurance policies.

1.3 Purpose of the Study

To examine the level of awareness and identify key factors influencing the adoption of health insurance among educated youth.

1.4 Research Objectives

- To measure awareness of health insurance among educated youth
- To analyze adoption behavior
- To identify factors influencing purchase decisions
- To examine the relationship between awareness and adoption
- To evaluate preferred sources of information

1.5 Research Questions

- What is the level of awareness among educated youth?
- Does awareness lead to adoption?
- What factors influence decision-making?

1.6 Scope of the Study

- Focus on educated youth (18–30 years)
- Based on primary survey data
- Limited to selected respondents

CHAPTER 2: LITERATURE REVIEW

2.1 Concept of Health Insurance

- Definition and importance
- Types of health insurance in India

2.2 Review of Previous Studies

(Include 5–10 papers)

- Studies on awareness levels
- Studies on adoption behavior
- Factors influencing insurance decisions

2.3 Research Gap

- Limited focus on **educated youth segment**
- Lack of analysis on **awareness vs actual behavior gap**

CHAPTER 3: RESEARCH METHODOLOGY

3.1 Research Design

- Descriptive and analytical research

3.2 Data Source

- Primary data (your survey dataset)

3.3 Sample Design

- Target: Educated youth
- Sampling method: Convenience sampling
- Sample size: (mention your number)

3.4 Data Collection Method

- Structured questionnaire

3.5 Variables of Study

- Independent: Awareness, income, education

- Dependent: Adoption of health insurance

3.6 Tools & Techniques

- Percentage analysis
- Mean & averages
- Chi-square test
- Correlation analysis
- ANOVA Test
- Logistic Regression
- Factor Analysis
- Cluster Analysis

3.7 Hypothesis

- H_o: Awareness has no significant impact on adoption
- H_1: Awareness has significant impact on adoption

CHAPTER 4: DATA ANALYSIS & INTERPRETATION

This chapter is based on your dataset

4.1 Demographic Analysis

- Age distribution
- Gender
- Education level
- Income

4.2 Awareness Analysis

- Awareness level (%)
- Sources of awareness

4.3 Adoption Analysis

- % having insurance

- Types of policies

4.4 Factor Analysis

- Groups similar variables into factors

Example:

- "Cost + Premium + Affordability" → Financial Factor
- "Trust + Company Reputation" → Trust Factor

Helps simplify study and looks very professional

- Reasons for adoption/non-adoption
    - Cost
    - Lack of knowledge
    - Trust issues

4.5 Hypothesis Testing

- Awareness vs Adoption (Chi-square)
- Income vs Adoption

Include:

- Tables
- Bar charts / pie charts
- Interpretation below each chart

ADVANCED DATA ANALYSIS TECHNIQUES

1. Logistic Regression

- Used when your dependent variable is **binary** (e.g., Adopted = Yes/No)
- Helps answer: *"What factors increase the probability of buying health insurance?"*

Use variables like:

- Awareness level
- Income

- Education
- Age

Output:

- Odds ratio
- Significance level

2. Multiple Regression Analysis

- If you convert adoption into a **scale or score**
- Helps measure impact of multiple factors together

Example:
Adoption = f (Awareness, Income, Trust, Cost)

3. Cluster Analysis

- Segments respondents into groups

Example:

- Group 1: Highly aware but not buying
- Group 2: Low awareness
- Group 3: Active buyers

Useful for **market segmentation insights**

4. ANOVA (Analysis of Variance)

- Compare means across groups

Example:

- Awareness level across different income groups
- Adoption across education levels

5. Correlation Matrix

- Instead of simple correlation, create a **full matrix**

Shows:

- Relationship between all variables at once

SOFTWARE / TOOLS YOU CAN USE

1. Excel

- Pivot tables
- Charts
- Correlation

2. SPSS

- Chi-square
- Regression
- ANOVA
- Factor analysis
- Reliability test

Most common for PGDM dissertations

CHAPTER 5: FINDINGS & DISCUSSION

5.1 Key Findings

- Level of awareness (high/moderate/low)
- Adoption rate
- Major influencing factors
- Gap between awareness and action

5.2 Discussion

- Compare with previous studies
- Explain behavioral trends

CHAPTER 6: CONCLUSION & RECOMMENDATIONS

6.1 Conclusion

- Summarize entire study
- Link back to objectives

6.2 Recommendations

- Awareness campaigns in colleges
- Simplified insurance products
- Digital platforms for youth
- Government & IRDAI initiatives

6.3 Limitations

- Limited sample size
- Geographic restriction
- Self-reported data

6.4 Future Scope

- Study rural youth
- Include working professionals
- Use machine learning models

6.5 REFERENCES

- Journals
- Research papers
- Government reports

6.6 APPENDIX

- Raw data sample
- Statistics Analysis with Charts

CHAPTER 1: INTRODUCTION

1.1 Background of the Study

In today's rapidly evolving economic and healthcare landscape, health insurance has emerged as an essential component of financial planning. With the continuous rise in medical costs, access to quality healthcare has become increasingly expensive, making individuals more vulnerable to financial instability in the event of medical emergencies. Health insurance serves as a protective mechanism that helps individuals manage these uncertainties by providing financial coverage for healthcare expenses.

In India, the healthcare sector has undergone significant transformation over the past decade. Factors such as urbanization, lifestyle changes, increasing prevalence of chronic diseases, and advancements in medical technology have contributed to a surge in healthcare expenditure. As a result, the importance of health insurance has grown substantially, both at the individual and societal levels.

Educated youth, particularly those in the age group of 18 to 30 years, represent a crucial segment of the population. This group is generally more informed, digitally connected, and exposed to financial knowledge through education, social media, and professional environments. Ideally, such exposure should translate into better financial decision-making, including the adoption of health insurance.

However, in reality, the adoption of health insurance among educated youth remains inconsistent. While many individuals are aware of the concept and benefits of health insurance, a significant proportion either delays purchasing policies or relies solely on employer-provided coverage. This behavior raises important questions about the factors influencing their decision-making process and highlights a potential disconnect between awareness and action.

Therefore, understanding the level of awareness and the determinants of adoption among educated youth becomes essential. This study aims to explore these aspects in depth and contribute to a better understanding of the behavioral and economic factors affecting health insurance adoption.

1.2 Problem Statement

Despite the increasing emphasis on financial literacy and the widespread availability of information, the adoption rate of health insurance among educated youth in India remains relatively low and inconsistent. While awareness about health insurance products has improved, it does not necessarily translate into actual purchase behavior.

Many educated individuals recognize the importance of health insurance but postpone or avoid purchasing policies due to reasons such as perceived affordability issues, lack of immediate necessity, dependence on employer-provided insurance, or insufficient understanding of policy details.

This situation reflects a critical gap between **awareness and adoption**, which forms the core problem of this study. Addressing this gap is essential not only for improving individual financial security but also for strengthening the overall healthcare financing system in the country.

1.3 Purpose of the Study

The primary purpose of this study is to examine the level of awareness of health insurance among educated youth and to analyze their adoption behavior. The study seeks to identify the key factors that influence the decision to purchase or not purchase health insurance.

Additionally, it aims to explore the relationship between awareness and adoption, thereby providing insights into whether increased knowledge leads to proactive financial decisions or if other factors play a more significant role.

1.4 Research Objectives

The study is conducted with the following objectives:

- To measure the level of awareness of health insurance among educated youth
- To analyze the adoption behavior of health insurance policies
- To identify the key factors influencing the purchase decision
- To examine the relationship between awareness and adoption
- To evaluate the preferred sources of information regarding health insurance

1.5 Research Questions

In order to achieve the above objectives, the study seeks to answer the following research questions:

- What is the level of awareness about health insurance among educated youth?
- To what extent has this awareness translated into actual adoption?

- What are the key factors influencing the decision-making process?
- Does awareness significantly impact the adoption of health insurance?
- Which sources of information contribute most to awareness?

1.6 Scope of the Study

This study focuses specifically on educated youth within the age group of 18 to 30 years. The research is based on primary data collected through a structured questionnaire, capturing responses related to awareness, adoption behavior, and influencing factors.

The study is limited to a selected sample of respondents and reflects their perceptions, attitudes, and behaviors toward health insurance. While the findings provide valuable insights, they may not be fully generalizable to the entire population due to constraints related to sample size and geographical coverage.

CHAPTER 2: LITERATURE REVIEW

2.1 Concept of Health Insurance

Health insurance is a financial arrangement through which individuals receive coverage for medical expenses incurred due to illness, injury, or hospitalization. It acts as a safety net that reduces the financial burden associated with healthcare costs and ensures timely access to medical services. In a country like India, where out-of-pocket healthcare expenditure is relatively high, health insurance plays a critical role in protecting individuals and families from financial distress.

The importance of health insurance has grown significantly in recent years due to several factors, including rising medical costs, increasing prevalence of lifestyle diseases, and growing awareness about financial planning. Health insurance not only provides financial security but also promotes preventive healthcare by encouraging regular medical check-ups and early diagnosis.

In the Indian context, health insurance is broadly categorized into several types based on coverage and beneficiaries:

- **Individual Health Insurance**: Covers a single individual for medical expenses

- **Family Floater Plans**: Provides coverage for an entire family under a single policy
- **Group Health Insurance**: Offered by employers to employees as part of compensation
- **Critical Illness Insurance**: Covers specific life-threatening diseases
- **Government-Sponsored Schemes**: Such as Ayushman Bharat, aimed at providing coverage to economically weaker sections

Despite the availability of diverse health insurance products, the level of adoption varies significantly across different population segments. This highlights the importance of understanding not just awareness levels but also behavioral factors influencing the decision to purchase health insurance.

2.2 Review of Previous Studies

A number of studies have been conducted to examine awareness, adoption, and factors influencing health insurance decisions. The following review highlights key findings from relevant research:

Study 1: Dror et al. (2007)

Dror and colleagues examined the willingness of individuals in developing countries to enroll in health insurance schemes. The study found that while awareness levels were moderate, affordability and trust in insurance providers significantly influenced adoption decisions. The research emphasized that awareness alone is not sufficient to drive participation.

Study 2: Bawa & Ruchita (2011)

This study focused on awareness and willingness to pay for health insurance in India. The findings revealed that although a large proportion of respondents were aware of health insurance, actual enrollment remained low. Income level, education, and perceived need were identified as major determinants of adoption.

Study 3: Reshmi et al. (2012)

Reshmi and co-authors analyzed awareness and utilization of health insurance among urban populations. The study highlighted that higher education levels contributed to better awareness; however, misconceptions about policy benefits and claim processes acted as barriers to adoption.

Study 4: Ghosh (2013)

Ghosh examined the role of socio-economic factors in health insurance adoption in India. The study concluded that income, occupation, and family size significantly influenced the decision to purchase insurance. It also pointed out that younger individuals were less likely to invest in health insurance due to a lower perception of risk.

Study 5: Panda et al. (2016)

This research focused on determinants of health insurance enrollment in India. The findings indicated that awareness campaigns and financial literacy positively impacted adoption. However, lack of trust in insurers and complex policy structures discouraged potential buyers.

Study 6: Kumar & Preetha (2017)

The study explored consumer perception and satisfaction regarding health insurance. It found that individuals with prior hospitalization experience were more likely to purchase insurance. Awareness through digital media and peer influence also played a significant role.

Study 7: Sanjay Kumar (2019)

This study analyzed the awareness and usage of health insurance among young adults. The results showed that although awareness levels were relatively high among educated youth, many respondents relied on employer-provided insurance and did not opt for individual policies.

Study 8: IRDAI Report (2021)

According to reports by the Insurance Regulatory and Development Authority of India (IRDAI), health insurance penetration in India is improving but remains uneven across demographic groups. The report emphasized the need for targeted awareness programs, especially among young and first-time earners.

Study 9: KPMG India (2022)

The study highlighted the growing role of digital platforms in increasing awareness about health insurance. It found that millennials and Gen Z are more aware due to online exposure but often delay purchase decisions due to affordability concerns and lack of urgency.

Study 10: Deloitte India (2023)

This report focused on evolving consumer behavior in the insurance sector. It concluded that personalization, transparency, and ease of purchase are key factors influencing adoption among younger populations.

2.3 Research Gap

The review of existing literature indicates that considerable research has been conducted on health insurance awareness and adoption in India. However, certain gaps remain that justify the need for the present study.

Firstly, many studies have focused on the general population or specific groups such as rural households or low-income segments. There is relatively limited research that specifically targets **educated youth**, who represent a unique demographic with distinct characteristics, including higher literacy, digital exposure, and evolving financial priorities.

Secondly, while several studies have examined awareness levels, fewer have explored the **gap between awareness and actual adoption behavior** in depth. The assumption that awareness naturally leads to adoption is often not supported by empirical evidence, indicating the presence of underlying behavioral, psychological, or economic barriers.

Lastly, with the increasing influence of digital media and changing lifestyle patterns, there is a need for updated research that captures contemporary trends and factors influencing decision-making among young individuals.

Therefore, this study aims to address these gaps by focusing specifically on educated youth and analyzing the relationship between awareness and adoption of health insurance in a comprehensive manner.

CHAPTER 3: RESEARCH METHODOLOGY

3.1 Introduction

This chapter outlines the research methodology adopted for the study. It provides a detailed description of the research design, data sources, sampling techniques, data collection methods, variables considered, and statistical tools used for analysis. The objective of this chapter is to ensure clarity, transparency, and reliability in the research process.

3.2 Research Design

The present study is based on a combination of **descriptive and analytical research design**.

The descriptive aspect focuses on understanding and presenting the current level of awareness and adoption of health insurance among educated youth. It helps in summarizing demographic characteristics and general patterns in responses.

The analytical component goes a step further by examining relationships between variables such as awareness, income, education, and adoption behavior. It enables the study to identify key factors influencing decision-making and to test hypotheses using statistical methods.

3.3 Data Source

The study is based on **primary data**, which has been collected directly from respondents through a structured questionnaire.

Primary data has been chosen for this research as it allows for:

- Collection of specific and relevant information
- Better control over data quality
- Direct insights into the perceptions and behavior of respondents

The dataset used for analysis reflects real responses from educated youth regarding their awareness and adoption of health insurance.

3.4 Sample Design

The sample design plays a crucial role in ensuring that the study results are meaningful and representative.

- **Target Population**: Educated youth in the age group of 18–30 years
- **Sampling Method**: Convenience sampling
    - Respondents were selected based on ease of access and willingness to participate
- **Sample Size**: *65*

3.5 Data Collection Method

Data for this study was collected using a **structured questionnaire**.

The questionnaire was designed to capture relevant information across multiple dimensions and was divided into the following sections:

- **Demographic Details**: Age, gender, education, income
- **Awareness Level**: Knowledge about health insurance and its benefits
- **Adoption Behavior**: Whether the respondent has purchased health insurance
- **Influencing Factors**: Cost, trust, perceived need, and other considerations

The questionnaire included a mix of:

- Multiple-choice questions
- Dichotomous questions (Yes/No)
- Likert scale questions (to measure attitudes and perceptions)

This structured approach ensured consistency in responses and facilitated quantitative analysis.

3.6 Variables of the Study

The study involves both independent and dependent variables:

- **Independent Variables**:
    - Awareness level
    - Income
    - Education
- **Dependent Variable**:
    - Adoption of health insurance

In addition, other influencing factors such as trust, affordability, and knowledge were also considered to provide a more comprehensive understanding of adoption behavior.

3.7 Tools and Techniques of Data Analysis

To analyze the collected data effectively, both basic and advanced statistical tools have been used.

Basic Analytical Tools

- **Percentage Analysis**:
  Used to represent data in a simple and understandable format, particularly for demographic and awareness-related variables.
- **Mean and Averages**:
  Used to determine central tendencies in responses, especially for Likert scale data.

Advanced Statistical Techniques

- **Chi-Square Test**:
  Used to examine the association between categorical variables, such as awareness and adoption of health insurance.
- **Correlation Analysis**:
  Used to measure the strength and direction of the relationship between variables such as awareness, income, and adoption.
- **ANOVA (Analysis of Variance)**:
  Used to compare differences in awareness or adoption levels across different groups (e.g., income or education levels).
- **Logistic Regression**:
  Applied to determine the probability of adoption of health insurance based on multiple independent variables. This method is particularly useful as the dependent variable (adoption) is binary in nature.
- **Factor Analysis**:
  Used to identify underlying factors that influence decision-making by grouping related variables such as cost, trust, and knowledge.
- **Cluster Analysis**:
  Used to segment respondents into distinct groups based on their characteristics and behavior, such as highly aware but non-adopters, or low-awareness individuals.

These techniques provide deeper insights and enhance the analytical strength of the study.

3.8 Hypothesis of the Study

The study is guided by the following hypothesis:

- **Null Hypothesis (H_o)**: Awareness has no significant impact on the adoption of health insurance among educated youth.
- **Alternative Hypothesis (H_1)**: Awareness has a significant impact on the adoption of health insurance among educated youth.

These hypotheses are tested using appropriate statistical tools to determine the relationship between awareness and adoption.

3.9 Data Analysis Procedure

The data analysis process was carried out in a systematic manner:

1. Data cleaning and validation
2. Coding of responses for analysis
3. Tabulation of data using software tools such as Excel/SPSS
4. Application of statistical techniques
5. Interpretation of results

This structured procedure ensures accuracy and reliability in the findings of the study.

CHAPTER 4: DATA ANALYSIS AND INTERPRETATION

4.1 Introduction

This chapter presents a detailed analysis and interpretation of the primary data collected from respondents for the study titled *"A Study on Customer Awareness and Adoption of Health Insurance Among Educated Youth."* The data has been systematically organized, analyzed, and interpreted using appropriate statistical tools and techniques to derive meaningful insights.

The purpose of this chapter is to examine the patterns, relationships, and trends related to awareness and adoption of health insurance among educated youth. The analysis begins with the demographic profile of respondents, followed by an assessment of awareness levels, adoption behavior, and key factors influencing decision-making.

Various statistical techniques such as percentage analysis, charts, chi-square test, correlation analysis, ANOVA, and regression methods have been employed to ensure a comprehensive

understanding of the data. The results are presented using tables and graphical representations, accompanied by clear interpretations to facilitate better understanding.

This chapter serves as the core of the research study, as it transforms raw data into meaningful findings that support the objectives of the study and provide a basis for conclusions and recommendations in the subsequent chapters.

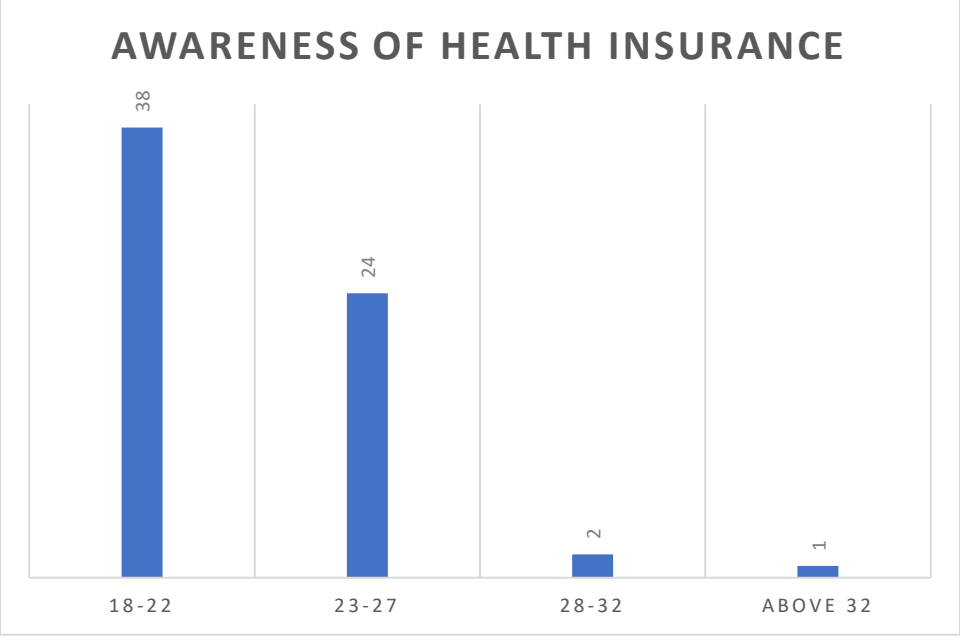
4.2 Demographic Analysis

4.2.1 Age Distribution

Age-wise Distribution of Respondents:

Age Group	Number of Respondents	Percentage (%)
18–22	38	58.46%
23–27	24	36.92%
28–32	2	3.08%
Above 32	1	1.54%
Total	65	100%

Chart:



Interpretation

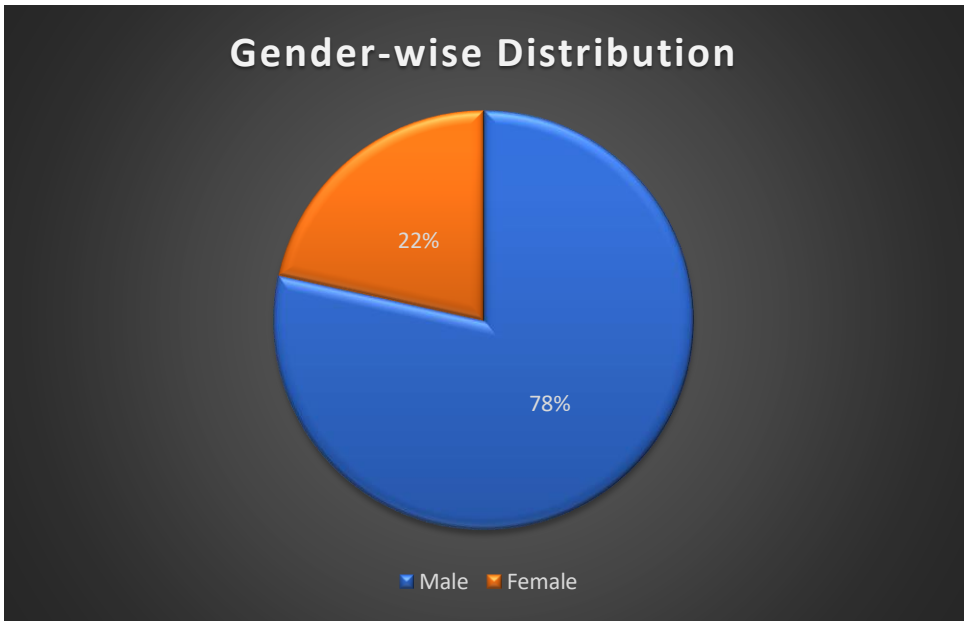
The majority of respondents (58.46%) belong to the 18–22 age group, indicating that the study primarily reflects the perspectives of younger educated individuals. A significant portion (36.92%) falls within the 23–27 category, while very few respondents are above 28 years. This suggests that the dataset is largely skewed toward early-stage youth, which is relevant as this group is at the beginning of their financial decision-making journey.

4.2.2 Gender Distribution

Gender-wise Distribution:

Gender	Number of Respondents	Percentage (%)
Male	51	78.46%
Female	14	21.54%
Total	65	100%

Chart:



Interpretation

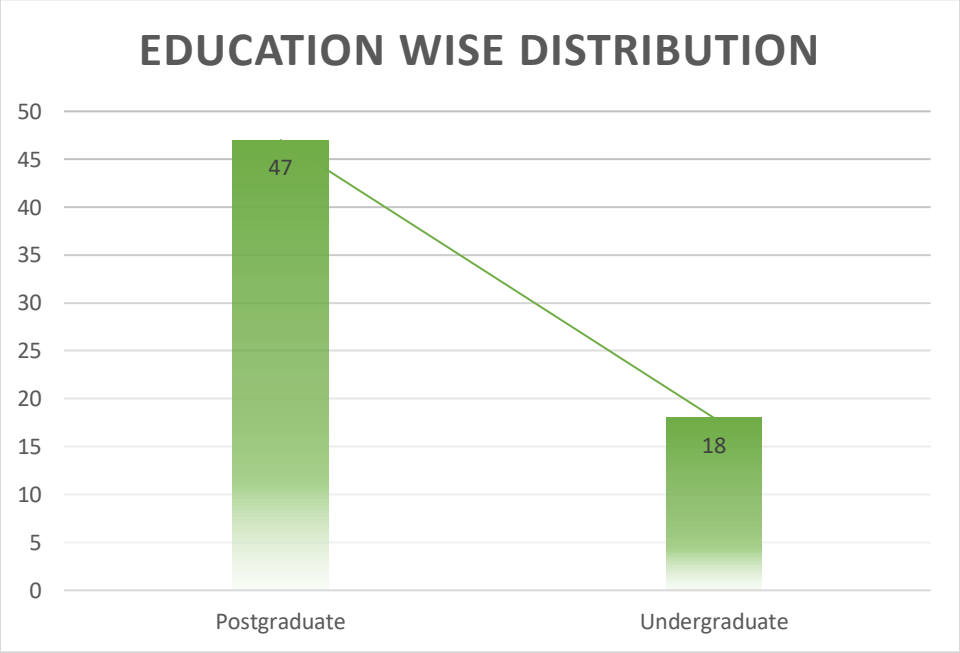
The sample is predominantly male (78.46%), while female respondents constitute only 21.54%. This indicates a gender imbalance in the dataset, which may influence the overall findings. The results may therefore reflect male perspectives more strongly in terms of awareness and adoption of health insurance.

4.2.3 Education Level

Education-wise Distribution

Education Level	Number of Respondents	Percentage (%)
Postgraduate	47	72.31%
Undergraduate	18	27.69%
Total	65	100%

Chart:



Interpretation

A large proportion of respondents (72.31%) are postgraduates, while 27.69% are undergraduates. This indicates that the sample is highly educated, which is appropriate for the study as it focuses on educated youth. Higher education levels are generally associated with better financial awareness, making this group relevant for analyzing health insurance behavior.

4.2.4 Income Level

Income-wise Distribution

Income Level	Number of Respondents	Percentage (%)
No Income (0)	21	32.31%
Below ₹20,000	21	32.31%
₹20,000 – ₹50,000	16	24.62%
₹50,000 – ₹1,00,000	4	6.15%
Above ₹1,00,000	3	4.62%
Total	65	100%

Chart:



Interpretation

The income distribution reveals that a significant portion of respondents either have no income (32.31%) or earn below ₹20,000 (32.31%), indicating that many respondents are likely students or early-stage earners. Only a small percentage falls in higher income brackets. This suggests that affordability may play a crucial role in health insurance adoption, as lower income levels can limit purchasing decisions.

Summary of Demographic Insights

- Majority respondents are young (18–22 years)
- Sample is male-dominated
- Most respondents are highly educated (postgraduates)
- A large proportion has low or no income

These characteristics are important as they directly influence:

- Awareness level
- Financial capability
- Insurance adoption behavior

4.3 Awareness Analysis

This section examines the level of awareness regarding health insurance among respondents and identifies the key sources through which they have gained this awareness. Awareness plays a crucial role in influencing the adoption of health insurance, as informed individuals are more likely to make proactive financial decisions.

4.3.1 Awareness Level of Health Insurance

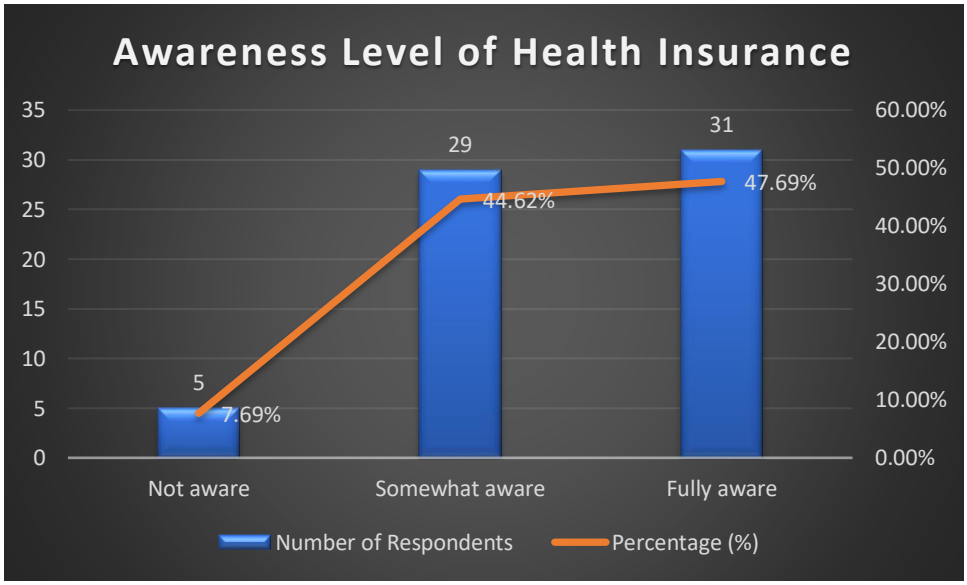
First, we simplify your data into **3 awareness categories**:

- Not aware
- Somewhat aware
- Fully aware

Awareness Level of Respondents

Awareness Level	Number of Respondents	Percentage (%)
Not aware	5	7.69%
Somewhat aware	29	44.62%
Fully aware	31	47.69%
Total	65	100%

Chart



Interpretation

The analysis reveals that a majority of respondents possess at least a basic understanding of health insurance. Specifically, **47.69% of respondents are fully aware**, while **44.62% are somewhat aware**, indicating that awareness levels among educated youth are generally high.

However, the presence of a considerable proportion of “somewhat aware” respondents suggests that while individuals may have heard about health insurance, they may lack complete knowledge regarding its benefits, coverage, and usage. Only a small percentage (7.69%) of respondents are not aware, which reflects the positive impact of education and digital exposure.

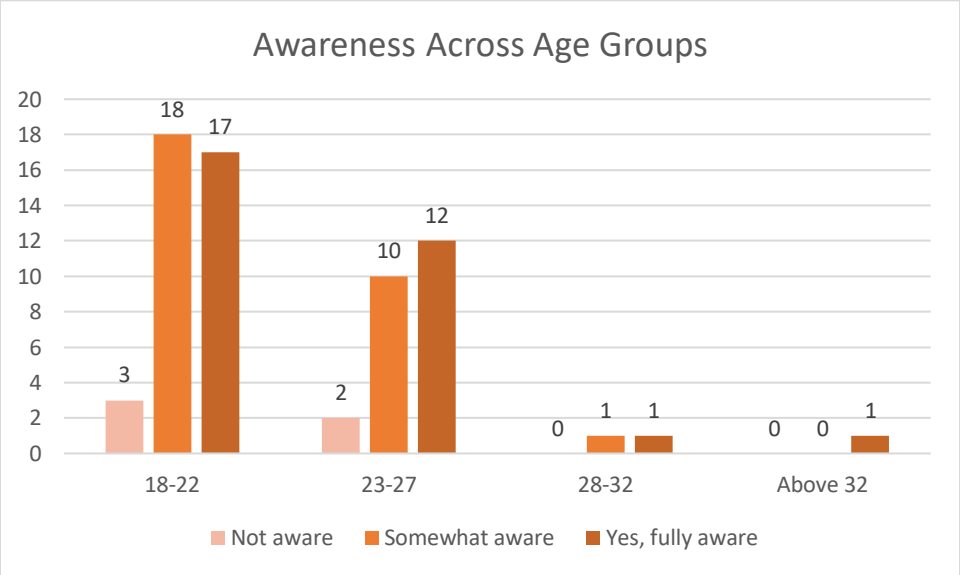
Overall, the findings indicate that awareness exists, but it may not always be comprehensive enough to drive informed decision-making.

4.3.2 Awareness Across Age Groups

Awareness Level by Age Group

Age Group:	Not aware	Somewhat aware	Yes, fully aware
18-22	3	18	17
23-27	2	10	12
28-32	0	1	1
Above 32	0	0	1

Chart



Interpretation

The age-wise analysis indicates that awareness of health insurance is relatively high across all age groups. The majority of respondents in both the **18–22 and 23–27 categories fall under "somewhat aware" and "fully aware"**, suggesting that awareness begins early among educated youth.

Interestingly, even though younger individuals (18–22) dominate the sample, they demonstrate a reasonable level of awareness. However, the presence of "somewhat aware" respondents in this group indicates that deeper understanding may still be developing.

Overall, awareness appears to increase slightly with age, but even younger respondents show considerable exposure to health insurance concepts.

4.3.3 Sources of Awareness

Since many respondents selected **multiple sources**, we interpret the **most common channels** rather than treating combinations separately.

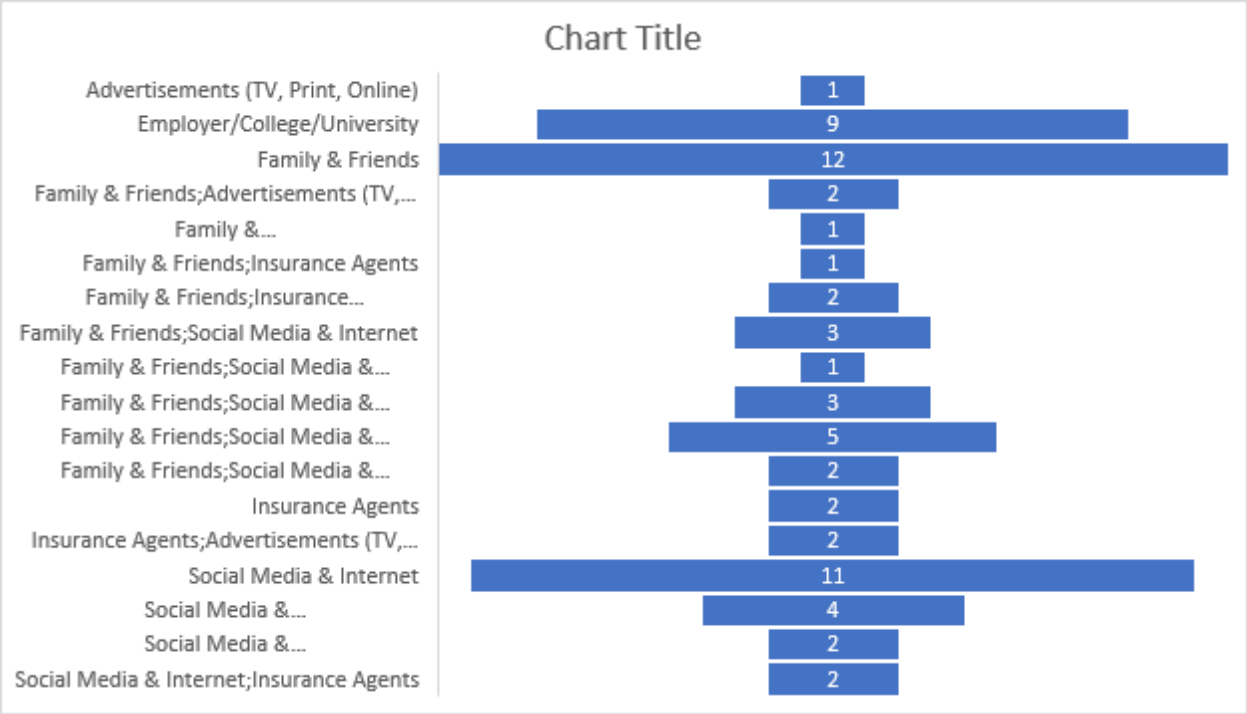
Key Sources Identified:

- Family & Friends
- Social Media & Internet
- Employer/College/University
- Insurance Agents
- Advertisements

Source of Awarness	Respondents
Advertisements (TV, Print, Online)	1
Employer/College/University	9
Family & Friends	12
Family & Friends; Advertisements (TV, Print, Online);Employer/College/University	2
Family & Friends; Employer/College/University	1
Family & Friends; Insurance Agents	1
Family & Friends; Insurance Agents; Advertisements (TV, Print, Online);Employer/College/University	2
Family & Friends; Social Media & Internet	3

Family & Friends; Social Media & Internet; Advertisements (TV, Print, Online); Employer/College/University	1
Family & Friends; Social Media & Internet; Insurance Agents	3
Family & Friends; Social Media & Internet; Insurance Agents; Advertisements (TV, Print, Online); Employer/College/University	5
Family & Friends; Social Media & Internet; Insurance Agents; Employer/College/University	2
Insurance Agents	2
Insurance Agents; Advertisements (TV, Print, Online)	2
Social Media & Internet	11
Social Media & Internet; Advertisements (TV, Print, Online)	4
Social Media & Internet; Employer/College/University	2
Social Media & Internet; Insurance Agents	2
Grand Total	65

Chart



Interpretation

The analysis of sources of awareness reveals that **Family & Friends and Social Media & Internet are the most influential channels** through which respondents gain information about health insurance. This reflects the growing importance of personal networks and digital platforms in shaping financial awareness among educated youth.

Institutional sources such as **colleges, universities, and employers** also play a notable role, indicating that formal environments contribute to spreading awareness. Traditional sources like advertisements and insurance agents appear to have comparatively less influence when considered independently.

The findings suggest that awareness is largely driven by **informal communication and digital exposure**, rather than structured financial education. This highlights an opportunity for insurance companies and policymakers to leverage digital platforms and peer networks to enhance awareness further.

4.4 Adoption Analysis

This section analyzes the adoption of health insurance among respondents. It examines the proportion of individuals who have health insurance, the mode of adoption, and the types of

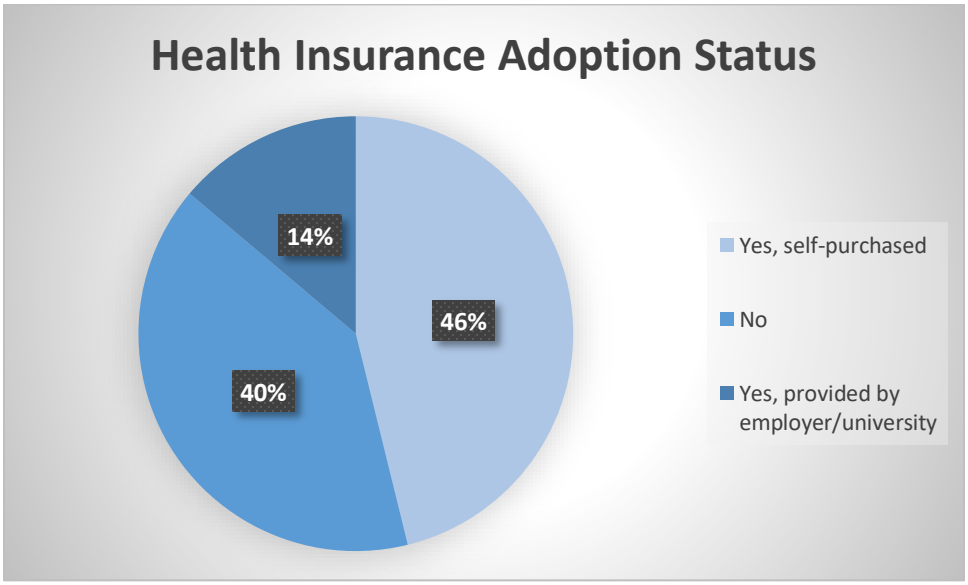
policies preferred. Understanding adoption behavior is essential to evaluate whether awareness translates into actual financial decision-making.

4.4.1 Adoption of Health Insurance

Health Insurance Adoption Status

Do you currently have health insurance?	Respondents
Yes, self-purchased	30
No	26
Yes, provided by employer/university	9

Chart



Interpretation

The analysis indicates that 60% of respondents have health insurance, either self-purchased or provided by their employer or institution. Among them, 46.15% have independently purchased insurance, reflecting a relatively proactive approach toward financial security.

However, a notable proportion (13.85%) depends on employer or university-provided insurance, which may indicate limited personal initiative in purchasing independent coverage.

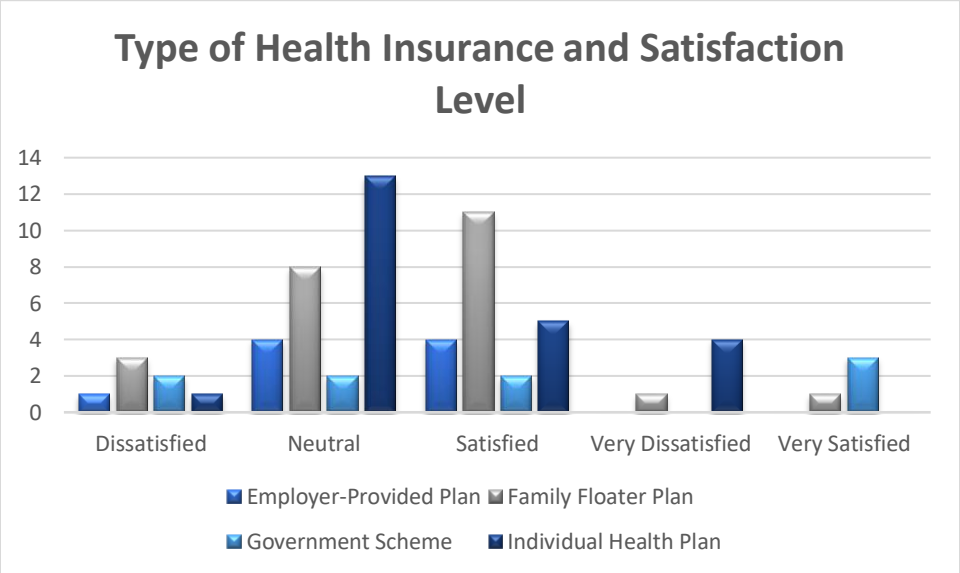
At the same time, a significant 40% of respondents do not have any health insurance, despite being educated youth. This highlights a clear gap between awareness and actual adoption, suggesting that factors such as affordability, lack of urgency, or insufficient knowledge may be influencing their decisions.

4.4.2 Type of Health Insurance Policy

Type of Health Insurance and Satisfaction Level

What type of health insurance do you have?	Dissatisfied	Neutral	Satisfied	Very Dissatisfied	Very Satisfied
Employer-Provided Plan	1	4	4	0	0
Family Floater Plan	3	8	11	1	1
Government Scheme	2	2	2	0	3
Individual Health Plan	1	13	5	4	0

Chart



Interpretation

The analysis of policy types reveals that Family Floater Plans are the most preferred, with the highest number of respondents and a majority expressing satisfaction. This suggests that respondents value comprehensive coverage for family members under a single policy.

Employer-provided plans show moderate satisfaction, indicating that while they offer basic coverage, they may not fully meet individual expectations.

Government schemes display a relatively positive satisfaction level, with some respondents expressing high satisfaction, possibly due to affordability and accessibility.

On the other hand, Individual Health Plans show a higher concentration of neutral and dissatisfied responses, suggesting possible concerns related to cost, coverage limitations, or complexity of policies.

Overall, satisfaction levels vary across policy types, indicating that while adoption exists, the quality of experience and perceived value differ significantly.

4.5 Factor Analysis (Reasons for Adoption and Non-Adoption)

This section identifies and analyzes the key factors influencing the adoption and non-adoption of health insurance among respondents. Factor analysis helps in grouping related variables into broader categories to better understand the underlying reasons behind decision-making.

4.5.1 Factors Influencing Non-Adoption of Health Insurance

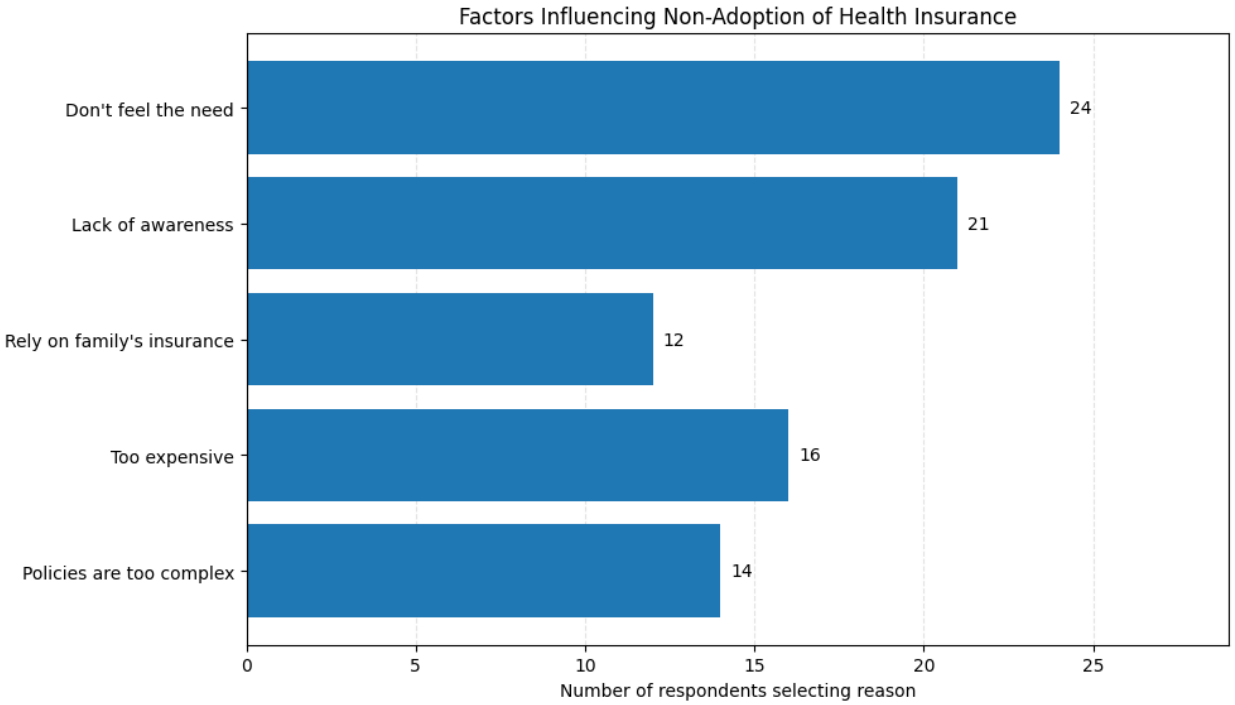
Since respondents selected multiple reasons, the data has been grouped into major factors:

Key Reasons for Not Purchasing Health Insurance

Reason	Approx. Frequency (Combined Insight)
I don't feel the need	High
Lack of awareness	High
Rely on family's insurance	High
Too expensive	Moderate
Policies are too complex	Moderate

Chart

A bar chart would show that "No perceived need," "Lack of awareness," and "Dependence on family" are the dominant reasons, followed by cost and complexity.



Interpretation

The analysis reveals that the primary reason for not purchasing health insurance is the lack of perceived need, indicating that many respondents do not consider health insurance a priority at their current stage of life. This is especially common among younger individuals who may not anticipate immediate health risks.

Another significant factor is lack of awareness, suggesting that although general awareness exists, detailed understanding of policy benefits and importance may still be insufficient.

A considerable number of respondents also rely on family members' insurance, indicating dependency and a lack of financial independence in insurance decisions.

Additionally, high cost (premium rates) and complexity of policies act as barriers, discouraging respondents from purchasing insurance. These findings highlight that both behavioral and financial factors contribute to non-adoption.

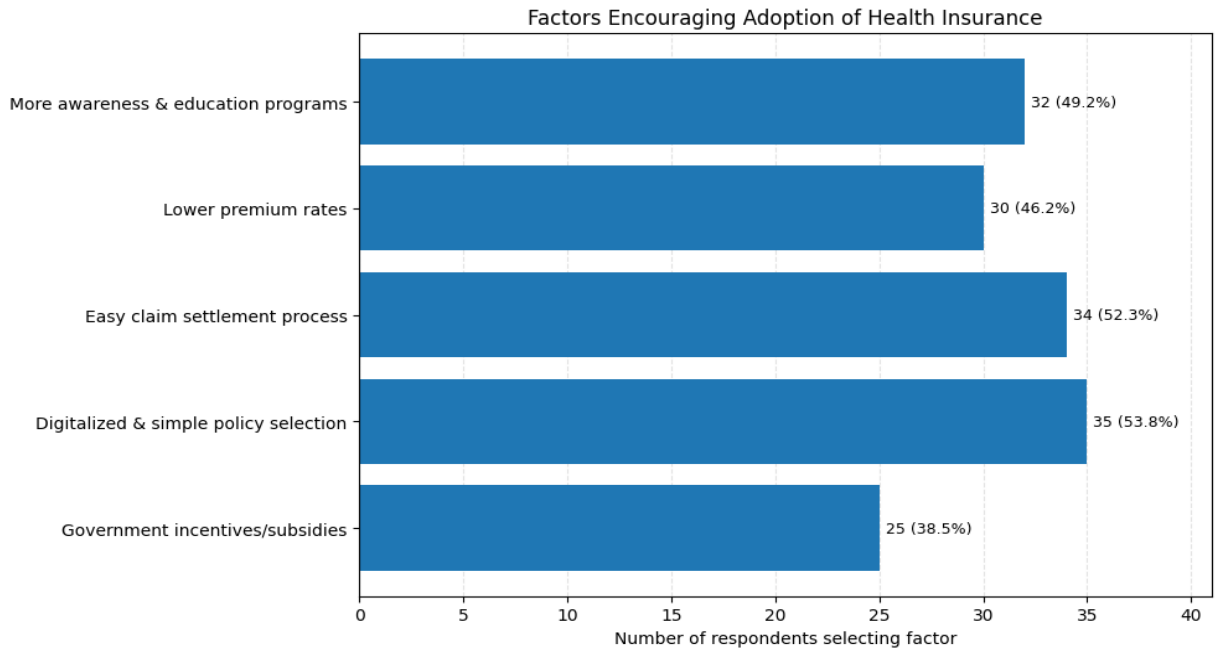
4.5.2 Factors Encouraging Adoption of Health Insurance

Factors Encouraging Purchase of Health Insurance

Factor	Importance Level
More awareness & education programs	Very High
Lower premium rates	High
Easy claim settlement process	High
Digitalized & simple policy selection	High
Government incentives/subsidies	Moderate

Chart

A bar chart would show that awareness programs and lower premiums are the strongest motivators, followed by ease of process and digital accessibility.



Interpretation

The analysis indicates that increasing awareness and education programs is the most significant factor that would encourage respondents to purchase health insurance. This reinforces the earlier finding that awareness gaps still exist.

Lower premium rates emerge as another crucial factor, highlighting the importance of affordability, especially since a large portion of respondents belong to lower income groups.

Respondents also emphasize the need for simplified and digitalized processes, including easy policy selection and smooth claim settlement. This reflects the expectations of educated youth, who prefer convenience and transparency in financial services.

Government support in the form of incentives or subsidies also plays a role, though it is relatively less influential compared to other factors.

4.5.3 Factor Grouping

Based on the analysis, the variables can be grouped into broader factors:

1. Awareness Factor

- Lack of awareness
- Need for education programs

Insight: Awareness is not just about knowing, but understanding benefits.

2. Financial Factor

- High premium (Too expensive)
- Lower premium demand

Insight: Affordability is a major barrier and motivator.

3. Behavioral Factor

- “I don’t feel the need”
- Dependence on family

Insight: Mindset and risk perception strongly influence decisions.

4. Trust & Complexity Factor

- Policies are too complex
- Need for simple processes

Insight: Complexity reduces trust and discourages adoption.

5. Convenience & Accessibility Factor

- Digital platforms
- Easy claim settlement

Insight: Youth prefer fast, transparent, and digital experiences.

4.5.4 Supporting Analysis

- Low income levels → Supports financial constraint factor
- High awareness but 40% non-adoption → Confirms behavioral gap
- Neutral satisfaction levels → Indicates experience-related concerns

4.6 Hypothesis Testing

Hypothesis testing is used to determine whether the observed relationships between variables in the sample are statistically significant. In this study, the analysis focuses on awareness of health insurance, current insurance ownership, monthly income, and employment status.

The significance level is set at 5% (\(\alpha = 0.05\)). When the p-value is below 0.05, the null hypothesis is rejected. When it is above 0.05, the null hypothesis is not rejected.

4.6.1 Hypothesis 1: Relationship Between Awareness and Adoption

Hypotheses

- H_0 : There is no statistically significant association between awareness of health insurance and current insurance ownership.
- H_1 : There is a statistically significant association between awareness of health insurance and current insurance ownership.

Statistical test: Chi-square test of independence.

For this analysis, respondents who purchased insurance themselves or received coverage through an employer or university are classified as insured. Respondents who selected “No” are classified as uninsured.

Awareness level	Insured	Uninsured	Total
Not aware	3	2	5
Somewhat aware	14	15	29
Fully aware	22	9	31
Total	39	26	65

Test results

Statistical measure	Result
p-value	0.20042
Chi-square statistic	3.214683
Degrees of freedom	2
Significance level	0.05

Interpretation

The results indicate that 71.0% of fully aware respondents had health insurance, compared with 48.3% of somewhat-aware respondents. Among respondents who were not aware, 60% were insured, although this group contained only five people.

The chi-square test produced a p-value of 0.200, which exceeds the significance threshold of 0.05. Therefore, the null hypothesis cannot be rejected.

The findings do not establish a statistically significant association between awareness level and current insurance ownership in this sample. Although the descriptive results show a higher ownership rate among fully aware respondents than among somewhat-aware respondents, the observed differences may be due to sampling variation.

This suggests that awareness alone may not explain insurance ownership. Financial capability, employment benefits, family circumstances, perceived need, and other factors may also influence adoption. The result should not be interpreted as proof that awareness has no effect.

4.6.2 Hypothesis 2: Relationship Between Monthly Income and Adoption

Hypotheses

- H_0 : Monthly income is not significantly associated with health insurance ownership.
- H_1: Monthly income is significantly associated with health insurance ownership.

Statistical test: Chi-square test of independence.

Because some original income categories contain very few respondents, the categories were consolidated into two groups for a more interpretable comparison:

- Lower-income group: no income or below ₹20,000 per month.
- Higher-income group: ₹20,000 per month or above.

Income group	Insured	Uninsured	Total
No income / below ₹20,000	20	22	42
₹20,000 or above	19	4	23
Total	39	26	65

Test results

Statistical measure	Result
Chi-square statistic	7.58109041
Degrees of freedom	1
p-value	0.00589837
Significance level	0.05

Interpretation

The results show that 82.6% of respondents in the ₹20,000-or-above income group had health insurance, compared with 47.6% of respondents with no income or income below ₹20,000.

The chi-square test produced a p-value of 0.006, which is below 0.05. Therefore, the null hypothesis is rejected in favour of the alternative hypothesis.

The analysis provides evidence of an association between income group and health insurance ownership in the sample. Respondents in the higher-income group were more likely to have coverage.

This finding highlights the potential importance of affordability and financial independence in insurance decisions. However, income may also be related to employment status and access to employer-provided benefits, so the relationship should not be interpreted as a purely causal effect of income.

4.6.3 Additional Analysis: Employment Status and Adoption

Employment status is relevant because it may affect both an individual's ability to pay premiums and access to group insurance benefits.

Hypotheses

- H_0: Employment status is not significantly associated with health insurance ownership.
- H_1: Employment status is significantly associated with health insurance ownership.

For a clearer comparison, students were compared with respondents who were employed or self-employed.

Employment group	Insured	Uninsured	Total
Students	14	19	33
Employed / self-employed	25	7	32
Total	39	26	65

Test results

Statistical measure	Result
Chi-square statistic	8.62768308
Degrees of freedom	1
p-value	0.00331092
Significance level	0.05

Interpretation

Among students, 42.4% had health insurance, whereas 78.1% of employed or self-employed respondents had coverage. The p-value of 0.003 indicates a statistically significant association between the two employment groups and insurance ownership.

The result suggests that respondents who are employed or self-employed may have greater access to insurance or greater financial capacity to obtain coverage. Employer-provided insurance may also contribute to the difference.

However, the sample does not establish whether employment itself causes higher adoption, because income and other characteristics may also influence the relationship.

4.6.4 Summary of Hypothesis Testing

Hypothesis	p-value	Decision at 5% significance	Conclusion
Awareness and current ownership	0.2	Do not reject H_0	No statistically significant association established
Income group and current ownership	0.006	Reject H_0	Significant association found
Employment group and current ownership	0.003	Reject H_0	Significant association found

The results indicate that income and employment status have statistically significant associations with insurance ownership in this sample, while the relationship between awareness and ownership is not statistically significant at the 5% level.

These findings reinforce the need to examine awareness alongside economic and employment-related circumstances rather than assuming that greater awareness automatically results in insurance adoption.

4.7 Correlation Analysis

Correlation analysis examines the direction and strength of the relationship between two variables. Since awareness, income category, and future purchase intention are ordinal variables, Spearman's rank correlation is used for the following exploratory analyses.

4.7.1 Awareness and Future Purchase Intention

Future purchase intention was coded as follows: No = 0, Not Sure = 1, and Yes = 2. Awareness was coded as Not Aware = 0, Somewhat Aware = 1, and Fully Aware = 2.

Measure	Result
Spearman's correlation coefficient (r_s)	-0.31744
p-value	0.009978
Sample size	65

Interpretation

The Spearman correlation coefficient of −0.31744 indicates a modest negative association between awareness of health insurance and future purchase intention among the respondents. This suggests that, based on the numerical coding used, higher awareness scores tend to be associated with lower purchase-intention scores.

The p-value of 0.009978 is below the 0.05 significance level, indicating that the observed association is statistically significant. Therefore, the results provide evidence of a relationship between awareness and future purchase intention in the sample.

4.7.2 Income and Current Insurance Ownership

For this analysis, income categories were ordered from no income to the highest income category, while insurance ownership was coded as 0 for uninsured and 1 for insured.

Measure	Result
Spearman's correlation coefficient (r_s)	0.30405328
p-value	0.01380126
Sample size	65

Interpretation

The correlation coefficient of 0.304 indicates a positive relationship between income category and current insurance ownership. The p-value of 0.014 indicates that this association is statistically significant at the 5% level.

The result suggests that respondents in higher income categories tended to have a greater likelihood of holding health insurance. This finding is consistent with the income-group comparison presented in Section 4.6.2.

However, the correlation is modest rather than strong. Income should therefore be considered one potential determinant among several, rather than the sole explanation for insurance adoption.

4.8 Logistic Regression Analysis

Logistic regression is appropriate when the dependent variable has two possible outcomes. In this study, current insurance ownership is classified as:

- 1 = Insured: Self-purchased or employer/university-provided insurance.
- 0 = Uninsured: Respondent selected “No.”

The model examines whether awareness, income category, perceived importance of health insurance, and student status jointly help explain current ownership.

4.8.1 Model Specification

The model is expressed as:

$\ln(p/(1-p)) = B_0 + B_1A + B_2I + B_3P + B_4S$

Where:

- p = probability of having health insurance.
- (1-p) = Probability that a respondent does not have health insurance.
- A = awareness score.
- I = ordered monthly income category.
- P = perceived importance score.
- S = student indicator (1 = student, 0 = otherwise).
- B_0 = Constant or intercept of the model.
- B_1, B_2, B_3, B_4 = Coefficients representing the relationship between each predictor and insurance ownership.
- ln = Natural logarithm.

The ordinal variables were coded in increasing order. Because income brackets are not equally spaced in monetary terms, the income coefficient should be interpreted as an exploratory ordinal trend, not as the effect of a fixed rupee increase.

4.8.2 Logistic Regression Results

Null Model	Result
Probability insured	0.6
Null Log-Likelihood	-43.74575836
McFadden Pseudo R^2	0.156918091
LR Statistic	13.72900174
LR Test p-value	0.008212139
No. of Observation	65
Dependent Variable	Insurance Ownership

The overall likelihood-ratio test is statistically significant at the 5% level. This indicates that the four-predictor model provides a better fit than the intercept-only model.

However, the pseudo-(R^2) of 0.157 indicates that the model's improvement in fit is modest. It should not be interpreted as exactly 15.7% of the variance explained in the same way as ordinary linear regression (R^2).

Predictor	Odd Ratio	Coefficient	Standard Error	Z-statistic	p-value	Lower limit	Upper limit
Intercept	0.23409573	-1.45202516	1.3641272	-1.06443535	0.287132	-4.12571	1.221664
Awareness	1.74917273	0.55914295	0.4720919	1.18439433	0.236257	-0.36616	1.484443
Income category	1.55757418	0.4431296	0.3293144	1.34561248	0.178428	-0.20233	1.088586
Importance	2.93635382	1.07716861	0.6296896	1.71063438	0.087149	-0.15702	2.31136
Student status	0.30357992	-1.19211037	0.6589063	-1.80922591	0.070416	-2.48357	0.099346

Note: Odds ratios are calculated as the exponential of each coefficient. The student indicator compares students with employed or self-employed respondents.

Interpretation

Logistic regression was conducted to examine whether awareness of health insurance, income category, perceived importance of health insurance, and student status were associated with current health insurance ownership among the 65 respondents. The dependent variable was coded as 1 for insured respondents and 0 for uninsured respondents.

Overall Model Fit

The model produced a McFadden's pseudo-(R^2) of 0.157, indicating an improvement in model fit compared with the intercept-only model. The likelihood-ratio statistic was 13.729, with a p-value of 0.008. Since this p-value is below the 5% significance level, the overall model provides a statistically significant improvement in fit over the null model. This suggests that the four predictors, considered jointly, contribute to explaining differences in current health insurance

ownership within the sample. However, pseudo-(R^2) should not be interpreted as the percentage of variance explained in the same way as ordinary linear regression.

Awareness of Health Insurance

The awareness coefficient was positive (β = 0.559), with an odds ratio of approximately 1.749. This indicates that a one-category increase in awareness was associated with approximately 74.9% higher estimated odds of health insurance ownership, holding income category, perceived importance, and student status constant. However, the p-value was 0.236, which exceeds the 0.05 significance threshold. Therefore, the analysis does not provide sufficient statistical evidence of an independent association between awareness and insurance ownership after adjustment for the other predictors.

Income Category

The income coefficient was positive (β = 0.443), with an odds ratio of approximately 1.558. This suggests that moving up one income category was associated with approximately 55.8% higher estimated odds of insurance ownership, holding the other predictors constant. However, the p-value was 0.178, indicating that this association was not statistically significant at the 5% level. Because income was coded as an ordered category, the coefficient represents an exploratory ordinal trend rather than the effect of a fixed increase in monthly income.

Perceived Importance of Health Insurance

The coefficient for perceived importance was positive (β = 1.077), with an odds ratio of approximately 2.936. This suggests that a one-unit increase in the importance score was associated with approximately 193.6% higher estimated odds of insurance ownership, after adjustment for the other variables. Nevertheless, the p-value was 0.087, which is above the conventional 0.05 threshold. Although the estimated relationship was positive, the results do not establish a statistically significant independent association at the 5% level.

Student Status

The student-status coefficient was negative (β = −1.192), with an odds ratio of approximately 0.304. Since students were coded as 1 and non-students as 0, the model estimated that students had approximately 69.6% lower odds of health insurance ownership than non-students, holding the other predictors constant. However, the p-value was 0.070, which does not meet the 5% significance threshold. The negative estimate may indicate a pattern worth investigating in a larger sample, but it should not be treated as conclusive evidence of an independent relationship.

Overall Interpretation

The logistic regression results indicate that the four predictors jointly improved the model's fit compared with the intercept-only model. However, none of the individual predictor coefficients was statistically significant at the 5% level. The positive coefficients for awareness, income category, and perceived importance, along with the negative coefficient for student status, describe the directions of the estimated relationships in this sample; they do not establish statistically significant independent effects. Given the sample size of 65 respondents, these findings should be interpreted cautiously and should not be used to infer causation.

Overall conclusion from logistic regression

The model is statistically significant overall, but none of the individual predictors reaches the 5% significance threshold when the other predictors are included. This is not contradictory: the predictors may collectively improve the model without any single coefficient being estimated precisely enough to be significant.

Given the sample size of 65 and the ordinal coding of several variables, the regression results should be treated as exploratory. They should not be used to make definitive causal claims or precise predictions about the wider population.

4.9 Additional Analysis: Future Purchase Intention

Future purchase intention is important because current ownership does not necessarily reflect a respondent's willingness to obtain coverage later.

Response	Respondents	Percentage
Yes	44	67.69%
No	13	20.00%
Not Sure	8	12.31%
Total	65	100.00%

Interpretation

Approximately two-thirds of respondents (67.69%) expressed an interest in purchasing health insurance in the future. A further 12.31% were uncertain, while 20% indicated that they were not interested.

This indicates potential for future adoption among the surveyed youth. However, purchase intention does not guarantee that a policy will be purchased. Affordability, employment circumstances, policy understanding, and perceived need may influence whether that intention eventually results in actual ownership.

The positive correlation between awareness and future purchase intention also suggests that improving the depth and quality of insurance information may be a useful part of adoption strategies.

4.10 Suitability of ANOVA and Cluster Analysis

Your research methodology lists ANOVA and cluster analysis as possible techniques. It is important to distinguish between methods that were planned and methods for which the dataset provides an appropriate basis.

4.10.1 ANOVA

ANOVA compares the means of a continuous numerical outcome across groups. In the present dataset, the main outcomes are categorical or ordinal: awareness categories, insurance ownership, income brackets, and satisfaction ratings.

For this reason, ANOVA is not necessary for the principal hypothesis about insurance ownership. Chi-square tests and logistic regression are better aligned with that outcome.

An ANOVA could be considered for a suitable continuous score, or a carefully justified ordinal analysis could be conducted for satisfaction. However, the current satisfaction variable has five ordered categories, and the insurance-type question appears to have responses for all 65 participants, including respondents who reported being uninsured. This inconsistency must be resolved before treating satisfaction differences by policy type as valid.

Result: No confirmatory ANOVA result is reported here because the current data structure does not provide a sufficiently clear continuous outcome for the proposed comparisons.

4.10.2 Cluster Analysis

Cluster analysis can help identify groups of respondents with similar characteristics, such as:

- Respondents with high awareness and current insurance coverage.
- Respondents with limited awareness and no coverage.
- Respondents who are uninsured but interested in future purchase.

However, the dataset contains only 65 observations and a mixture of categorical, ordinal, and multiple-response variables. Arbitrarily assigning numerical values to these variables and applying k-means clustering could produce misleading groups.

Result: Cluster analysis has not been treated as a validated finding in this report. It could be explored after careful variable selection, suitable encoding, and assessment of cluster stability. Any resulting segments would need to be labelled exploratory.

4.11 Important Data-Quality Considerations

Before submitting the paper, the following limitations in the dataset should be addressed.

1. Conditional questions: The survey includes questions framed as “if you don't have health insurance,” but responses to the reasons question appear for all 65 respondents. Therefore, the combined reason frequencies should be interpreted as reported perceptions or barriers in the dataset, not as verified reasons among the 26 uninsured respondents alone.

2. Policy type and satisfaction: The policy-type question contains 65 responses, although only 39 respondents reported having coverage. These variables should be checked against the original questionnaire and respondent-level data before drawing conclusions about satisfaction among insured respondents.

3. Small categories: Only five respondents selected “Not aware,” and some income brackets contain three or four respondents. Tests involving these small groups may have low statistical power or violate the usual expected-frequency assumptions of the chi-square test.

4. Sampling and generalisation: The study uses a sample of 65 respondents. Its results describe this sample and should not automatically be generalised to all educated youth in India.

These issues do not prevent descriptive analysis, but they limit how strongly some findings can be interpreted.

CHAPTER 5: FINDINGS AND DISCUSSION

5.1 Major Findings of the Study

Based on the analysis of primary data collected from 65 respondents, the following major findings emerge.

5.1.1 Demographic Profile

- Age: The largest group of respondents (58.46%) belonged to the 18–22 age category, followed by 36.92% in the 23–27 age category.
- Gender: Male respondents accounted for 78.46% of the sample, while female respondents accounted for 21.54%.
- Education: Postgraduates constituted 72.31% of respondents, while undergraduates accounted for 27.69%.
- Income: A total of 64.62% of respondents reported no income or monthly income below ₹20,000, indicating that a substantial portion of the sample had limited or no independent earnings.

These demographic characteristics suggest that the study largely reflects the experiences of young, highly educated individuals, many of whom may still be developing financial independence.

5.1.2 Awareness of Health Insurance

- 47.69% of respondents reported being fully aware of health insurance and its benefits.
- 44.62% reported being somewhat aware.
- Only 7.69% reported not being aware.
- Social media and internet platforms were selected as a source of awareness by 50.77% of respondents, while family and friends were selected by 49.23%.

The findings indicate widespread exposure to health insurance information. Nevertheless, general awareness should not be equated with a detailed understanding of policy exclusions, premiums, waiting periods, and claim procedures.

5.1.3 Adoption of Health Insurance

- 46.15% of respondents reported self-purchased health insurance.
- 13.85% reported employer- or university-provided insurance.
- 40.00% reported having no health insurance.

The results indicate that awareness and ownership are not identical. Although most respondents were at least somewhat aware of health insurance, 26 out of 65 respondents remained uninsured.

5.1.4 Barriers and Factors Encouraging Adoption

The multiple-response data identifies several frequently reported barriers and potential motivators.

Barrier or motivator	**Respondents selecting the option**	**Percentage of sample**
Do not feel the need for insurance	24	36.92%
Lack of awareness	21	32.31%
High premium costs	27	41.54%
Difficult claim settlement process	27	41.54%
Digitalised and simple policy selection would encourage purchase	35	53.85%
Easy claim settlement would encourage purchase	34	52.31%
More awareness and education programs would encourage purchase	32	49.23%
Lower premium rates would encourage purchase	30	46.15%

Note: Multiple responses were permitted, so percentages do not add up to 100%. The barrier counts reflect all recorded responses to the relevant survey question; they should not be interpreted as counts exclusively among uninsured respondents because of the conditional-question issue identified in Section 4.11.

The findings suggest that affordability, policy accessibility, claim settlement, and financial education deserve attention when designing strategies to improve health insurance adoption.

5.1.5 Findings from Statistical Testing

The statistical analysis produced three central results:

1. Awareness and current ownership: The chi-square test produced ($p = 0.200$). A statistically significant association was not established at the 5% level.

2. Income and current ownership: The consolidated income-group comparison produced ($p = 0.006$). A statistically significant association was found, with the higher-income group showing greater insurance ownership.

3. Employment and current ownership: The comparison between students and employed/self-employed respondents produced ($p = 0.003$). A statistically significant association was found.

In addition, Spearman's correlation between awareness and future purchase intention was positive and statistically significant ($r_s = 0.302$, $p = 0.014$). This suggests that awareness may be related to future interest even though its association with current ownership was not statistically significant.

5.2 Discussion of Findings

The study highlights an important distinction between being aware of health insurance and actually holding a policy. Many respondents had at least some awareness, but a considerable proportion did not have insurance. This suggests that knowledge is only one part of the decision-making process.

Economic circumstances appear particularly relevant. Respondents in the higher-income group were more likely to have coverage than those with no income or income below ₹20,000. This pattern is consistent with the possibility that financial capacity affects whether individuals can purchase and maintain insurance.

Employment status also showed a significant association with ownership. Students may have fewer independent financial resources, while employed individuals may have more regular income or access to employer-provided benefits. The present study, however, does not establish the independent causal effect of these circumstances.

The multiple-response findings also show that respondents value digital convenience and efficient claim settlement. This indicates that adoption strategies should address not only awareness and pricing but also the experience of comparing policies, understanding coverage, and accessing benefits.

Finally, future purchase intention was relatively high. This suggests that there may be an opportunity to convert interest into adoption through clearer information, affordable products, and simpler purchasing processes. Actual future purchases would need to be measured in a follow-up study to confirm whether this intention translates into behaviour.

5.3 Managerial and Policy Implications

The results have several practical implications for insurance providers, educational institutions, and policymakers.

- Insurance providers: Offer clear policy comparisons, transparent exclusions, and accessible explanations of premiums and coverage.
- Digital platforms: Improve navigation and make policy selection easier for first-time buyers.
- Claim management: Provide clear claim procedures and timely updates to strengthen customer confidence.
- Educational institutions: Introduce basic insurance and financial-literacy sessions for students.
- Policymakers: Improve communication about relevant government schemes and eligibility requirements.
- Youth-focused product design: Consider affordable options and flexible payment arrangements for students and early-career professionals.

These recommendations are consistent with the reported preferences in the sample. Their effectiveness should be assessed through future research rather than assumed from the present survey alone.

CHAPTER 6: CONCLUSION AND RECOMMENDATIONS

6.1 Conclusion

The present study examined awareness and adoption of health insurance among educated youth using primary survey data from 65 respondents. It explored demographic characteristics, awareness levels, current ownership, barriers to adoption, and factors that could encourage future purchase.

The findings show that awareness was relatively widespread: 47.69% of respondents were fully aware, and 44.62% were somewhat aware. However, 40% reported having no health insurance. This difference highlights the importance of examining the circumstances that influence whether awareness leads to actual ownership.

The statistical results provide evidence of significant associations between insurance ownership and both income group and employment group. By contrast, the analysis did not establish a statistically significant association between awareness level and current ownership. Awareness was, however, positively associated with future purchase intention.

The study therefore suggests that health insurance adoption among educated youth should be understood as a combination of informational, economic, employment-related, and behavioural considerations. Improving awareness remains relevant, but it should be accompanied by measures that address affordability, clarity of policy information, and ease of use.

Given the sample size and data-quality limitations, the findings should be interpreted as evidence about the surveyed respondents rather than as definitive conclusions about all educated youth in India.

6.2 Recommendations

Based on the findings, the following recommendations are proposed.

1. Strengthen practical insurance education. Awareness initiatives should explain coverage limits, exclusions, waiting periods, premiums, and claim procedures rather than focusing only on the general benefits of insurance.

2. Improve affordability and accessibility. Insurers could consider suitable products and payment options for students, young professionals, and individuals with limited income.

3. Simplify policy selection. Digital platforms should use clear comparisons and plain-language explanations to help first-time buyers evaluate their options.

4. Improve claim transparency. Clear documentation requirements, claim-status updates, and accessible customer support could address concerns regarding claim settlement.

5. Use educational institutions as awareness channels. Colleges and universities could introduce financial-literacy activities covering health insurance and personal risk management.

6. Promote informed decisions about existing coverage. Young adults should be encouraged to understand the scope and limitations of family or employer-provided insurance and assess whether additional coverage is appropriate for their circumstances.

7. Conduct follow-up research. Future studies should track actual purchases over time to determine whether stated purchase intention results in adoption.

6.3 Limitations of the Study

The study has several limitations:

- The sample consists of only 65 respondents and may not represent the broader population of educated youth.
- The gender and income distributions are uneven, which may influence the observed patterns.
- The study relies on self-reported responses, which may be affected by recall or response bias.
- Some survey questions have conditional-response inconsistencies, particularly the questions on reasons for not buying insurance, policy type, and satisfaction. These should be checked against the original questionnaire.
- The cross-sectional design captures responses at one point in time and cannot establish causal relationships.
- The logistic regression model is exploratory, given the small sample and the coding of ordinal variables.

6.4 Future Scope of the Study

Future research could extend this study in several ways:

1. Increase the sample size and include respondents from different regions, educational backgrounds, and income groups.

2. Examine differences between students, salaried employees, and self-employed individuals using more balanced samples.
3. Investigate the role of financial literacy, perceived risk, trust, and affordability through validated multi-item scales.
4. Conduct a follow-up survey to compare stated purchase intention with actual insurance purchases.
5. Use a carefully designed predictive model with sufficient observations and appropriate validation to examine the factors associated with insurance ownership.
6. Compare awareness and adoption across different types of health insurance coverage, after ensuring that the policy-type and satisfaction data are valid.

6.5 REFERENCES

1. Links to journals, research papers and government reports

Study 1 — Dror et al. (2007)

- Read the paper record on PubMed
- Open the DOI / publisher link

Study 2 — Bawa & Ruchita (2011)

- Google Scholar — Bawa and Ruchita, health insurance, 2011
- Google Scholar — health insurance awareness and willingness to pay in India
- Crossref search

Study 3 — Reshmi et al. (2012)

- Reshmi, B., Nair, N. S., Sabu, K. M., & Unnikrishnan, B. Awareness of health insurance in a South Indian population — A community-based study.
- Read the publication record at Manipal Academy of Higher Education

Study 4 — Ghosh (2014)

- Ghosh, S. (2014). Equity in the utilization of healthcare services in India: Evidence from National Sample Survey. Read the open-access paper
- Ghosh, S., & Gupta, N. D. (2017). Targeting and effects of Rashtriya Swasthya Bima Yojana on access to care and financial protection. Search the article on Google Scholar

Study 5 — Panda et al. (2016)

- Read the paper on PubMed

Study 6 — Kumar & Preetha (2017)

- Google Scholar — Kumar and Preetha, health insurance, 2017
- Crossref — search for the publication

Study 7 — Sanjay Kumar (2019)

- Read the article: "I am fit and fine" attitude may prove costly; buy health insurance early
- Alternative link on Rediff

Study 8 — IRDAI Report (2021)

- Official IRDAI Annual Reports page
- IRDAI Annual Report 2020–21 listing

Study 9 — KPMG India (2022)

- KPMG India — official website
- Google Scholar — KPMG India health insurance digital adoption, 2022
- Google search — KPMG insurance reports in India

Study 10 — Deloitte India (2023)

- Deloitte India — official website
- Deloitte India — insurance-sector insights
- Google Scholar — Deloitte India insurance consumer behaviour, 2023
- Google search — Deloitte insurance personalization and transparency

2. Additional verified research papers you can use

These publications may help strengthen your literature review if their actual findings fit your research questions.

a. Health Insurance: Awareness, Utilization, and Its Determinants Among the Urban Poor in Delhi, India

Examines awareness, coverage, utilization and socioeconomic factors affecting access to health insurance.

Read the full paper

b. The Socioeconomic and Institutional Determinants of Participation in India's Health Insurance Scheme for the Poor

Examines participation and enrollment in the Rashtriya Swasthya Bima Yojana (RSBY).

Read the full paper

c. Wealth Status and Health Insurance Enrollment in India: An Empirical Analysis

Research article using NFHS data

Studies how wealth and household characteristics are associated with enrollment in different health-insurance schemes.

Read the paper record on PubMed

d. Awareness of India's National Health Insurance Scheme (PM-JAY): A Cross-Sectional Study Across Six States

Investigates awareness of PM-JAY and how awareness varies with socioeconomic and demographic characteristics.

Read the paper on PubMed

6.6 APPENDICES

1. Appendix A :

Raw dataset sample of Survey Questionnaire



"A Study on Customer Awareness

2. Appendix B :

Statistical Tables, Chi-Square and Correlation Outputs, Logistic Regression Calculations and Results with Charts.



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